PHARMACOVIGILANCE INTERNSHIP

ASSIGNMENT 05: INDIVIDUAL CASE SAFETY REPORT (ICSR)

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1. INTRODUCTION TO PHARMACOVIGILANCE

Pharmacovigilance is the science and activities concerned with the detection, assessment, understanding and prevention of adverse effects or any other medicine-related problems.

Its primary objective is to ensure the safety of medicines throughout their life cycle, from clinical development to post-marketing use.

Pharmacovigilance involves collecting and analysing information about adverse drug reactions (ADRs), identifying potential safety signals and taking appropriate actions to minimise risks to patients.

An important component of pharmacovigilance is the Individual Case Safety Report (ICSR), which documents an individual patient's experience of a suspected adverse event following exposure to a medicinal product.

2. DEFINITION OF INDIVIDUAL CASE SAFETY REPORT (ICSR)

An Individual Case Safety Report (ICSR) is a structured report containing information about one identifiable patient who has experienced one or more suspected adverse reactions or adverse events associated with the use of one or more medicinal products.

An ICSR is used to communicate important safety information to pharmaceutical companies, regulatory authorities and other relevant stakeholders.

It provides essential information about:

- Patient demographics and medical history.
- Suspected medicinal products.
- Adverse events or reactions.
- Date of drug administration and event occurrence.
- Seriousness and severity of the event.
- Causality assessment.
- Treatment provided and patient outcome.

Purpose of an ICSR

1. To document suspected adverse drug reactions.
2. To identify potential safety concerns associated with medicines.
3. To evaluate the relationship between a medicinal product and an adverse event.
4. To support regulatory reporting requirements.
5. To improve patient safety through continuous monitoring of medicines.

3. SOURCES OF ICSR REPORTS

Adverse event reports can originate from different sources. These are broadly classified into two categories.

3.1 Solicited Sources

Solicited reports are obtained through organised data collection systems or structured activities.

Examples include:

- Clinical trials.
- Patient support programmes.
- Compassionate use programmes.
- Post-authorisation safety studies.
- Registries and organised patient follow-up programmes.

3.2 Unsolicited Sources

Unsolicited reports are voluntarily submitted by individuals or healthcare professionals without being requested through an organised data collection programme.

Examples include:

- Spontaneous reports from doctors.
- Reports from pharmacists and nurses.
- Patient or consumer reports.
- Medical literature.
- Reports received through pharmaceutical company websites or other spontaneous reporting channels.

Both types of sources can contribute important safety information and help identify previously unrecognised adverse reactions.

4. MINIMUM CRITERIA FOR A VALID ICSR

Before processing an adverse event report, the Drug Safety Associate must confirm that it meets four minimum validity criteria.

MINIMUM CRITERION	EXPLANATION
Identifiable patient	Information sufficient to distinguish the patient, such as initials, age, gender or patient identification details
Identifiable reporter	An identifiable person who provides the information, such as a doctor, pharmacist or patient

Suspected medicinal product	At least one identifiable medicine suspected of causing the event
Suspected adverse event/reaction	At least one identifiable adverse event or reaction

All four criteria must be available for the report to qualify as a valid ICSR.

If any information is missing, the associate should attempt to obtain it through follow-up. The information should still be documented and retained according to the applicable company procedure.

5. DAY 0 AND REGULATORY REPORTING TIMELINES

5.1 Meaning of Day 0

Day 0 is the date on which the pharmaceutical company or marketing authorisation holder first receives information that fulfils the applicable criteria for a report.

It is also known as the initial receipt date or clock start date.

Day 0 is important because regulatory reporting timelines are calculated from the applicable clock start date.

For example, if a company receives a valid serious adverse event report on 10 June 2026, that date is recorded as Day 0, subject to the applicable reporting rules.

5.2 Regulatory Reporting Timelines

Reporting timelines depend on the type of report, the source of information, the applicable regulations and the jurisdiction.

The training video mentions the following general timelines:

Timeline	Type of case mentioned in the tutorial
7 days	Fatal or life-threatening cases
15 days	Other serious cases
90 days	Non-serious cases

These timelines should not be considered universal requirements for every post-marketing ICSR.

For example, under the EU post-marketing reporting framework, serious valid ICSRs generally have a 15-day reporting timeline and non-serious valid ICSRs have a 90-day timeline. In India, applicable CDSCO requirements and the relevant product-specific reporting framework must be followed. The CDSCO 2024 PSUR guidance specifies reporting of serious adverse events within 15 calendar days and non-serious adverse events within 90 calendar days for the covered MAH reporting requirements.

Fatal or life-threatening suspected unexpected adverse reactions in clinical trials may have a separate 7-day expedited reporting requirement, with additional follow-up information. Therefore, the Drug Safety Associate must always verify the relevant regulatory requirement and company SOP before submission.

6. ICSR CASE PROCESSING WORKFLOW

ICSR processing involves multiple stages to ensure that adverse event information is accurately documented, medically evaluated and submitted to the appropriate regulatory authority.

6.1 Case Receipt and Triage

Triage is the initial stage of case processing.

The Triage Associate receives the adverse event report and performs the following activities:

- Reviews the information received.
- Checks the four minimum ICSR validity criteria.
- Identifies the initial receipt date or Day 0.
- Determines the preliminary seriousness of the case.
- Searches the safety database for possible duplicate reports.
- Prioritises cases according to applicable regulatory timelines.
- Identifies missing information and initiates follow-up queries.

A report that meets all four minimum validity criteria can proceed for ICSR processing.

6.2 Data Entry

After successful triage, the case moves to the data entry stage.

The Data Entry Associate enters the available information into the company's safety database.

Commonly used pharmacovigilance databases include:

- Oracle Argus Safety.
- ArisGlobal LifeSphere Safety.
- Other validated pharmacovigilance safety systems.

The major responsibilities include:

1. Entering patient demographic information.
2. Recording medical history and relevant clinical information.
3. Entering suspected and concomitant medication details.
4. Recording adverse events and their onset dates.
5. Documenting treatment and patient outcomes.
6. Coding medical terms using MedDRA.
7. Coding medicinal products using the WHO Drug Dictionary or an approved company dictionary.
8. Preparing the case narrative.
9. Raising queries for missing or unclear information.

6.3 Medical Coding

Medical coding involves converting medical information into standardised terminology for consistent documentation and analysis.

A. MedDRA

The Medical Dictionary for Regulatory Activities (MedDRA) is an internationally recognised medical terminology used for coding adverse events, medical history, indications and other relevant medical information.

MedDRA is organised into five hierarchical levels:

1. System Organ Class (SOC).
2. High Level Group Term (HLGT).
3. High Level Term (HLT).
4. Preferred Term (PT).
5. Lowest Level Term (LLT).

B. WHO Drug Dictionary

The WHO Drug Dictionary is used to standardise medicinal product information, including drug names, active ingredients and relevant product details.

Correct medical and drug coding helps ensure consistency in pharmacovigilance databases and regulatory submissions.

6.4 Case Narrative Preparation

A case narrative is a concise chronological summary of the patient's experience.

It generally includes:

- Patient demographics.
- Relevant medical history.
- Suspected medicinal product and dosage.
- Date of treatment initiation.
- Description of the adverse event.
- Date of event onset.
- Medical investigations and treatment.
- Drug withdrawal or dose modification.
- Patient outcome.
- Causality and seriousness assessment, where available.

The narrative should be clear, objective, chronological and based on the information provided in the source report.

6.5 Quality Review

Quality review is performed to ensure that the information entered into the safety database is complete and accurate.

The Quality Reviewer checks:

- Accuracy of patient information.
- Correct entry of suspected and concomitant medications.
- Accuracy of event descriptions and dates.
- Appropriate MedDRA coding.
- Appropriate drug coding.
- Completeness and readability of the case narrative.
- Consistency between the source document and database entries.
- Correct documentation of seriousness and outcome.

If discrepancies or missing information are identified, corrections or follow-up queries are initiated.

6.6 Medical Review

Medical review is generally performed by a qualified physician or appropriately authorised medical reviewer.

The reviewer evaluates:

- Clinical relevance of the adverse event.
- Relationship between the suspected drug and the event.
- Seriousness of the reaction.
- Clinical severity.
- Relevant medical history and alternative causes.
- Patient outcome.
- Accuracy of medical coding.
- Need for additional information.

The medical reviewer may also provide a company causality assessment and medical comments.

6.7 Regulatory Submission

The final stage involves submitting the completed ICSR to the appropriate regulatory authority or safety reporting system.

Before submission, the Submission Associate verifies:

- Case completeness.
- Validity of the report.
- Correctness of coding.
- Appropriate reporting classification.
- Compliance with the applicable reporting deadline.
- Correct electronic submission format.

ICSRs are commonly transmitted using the ICH E2B electronic reporting standard.

After submission, the acknowledgement and any validation errors should be monitored and documented.

7. PHARMACOVIGILANCE ASSESSMENT OF AN ICSR

A complete pharmacovigilance assessment involves evaluating the adverse event and determining its possible relationship with the suspected medicinal product.

7.1 ADR Identification

An adverse drug reaction is a harmful and unintended response to a medicinal product for which there is at least a reasonable possibility of a causal relationship with the medicine.

The Drug Safety Associate must identify:

- Signs and symptoms reported by the patient.
- Clinical diagnosis provided by the physician.
- Relevant laboratory abnormalities.
- Time of onset.
- Suspected medication.
- Possible alternative causes.

7.2 Causality Assessment

Causality assessment is the process of evaluating whether an adverse event is potentially related to the administration of a medicinal product.

One commonly used method is the WHO-UMC causality assessment system.

The categories include:

Category	Meaning
Certain	Strong evidence of a causal relationship with a plausible time relationship and no reasonable alternative explanation
Probable/Likely	Reasonable time relationship, alternative causes are unlikely, and response to drug withdrawal is clinically reasonable
Possible	Reasonable time relationship, but other causes may also explain the event
Unlikely	Time relationship makes a causal association improbable, although it cannot be completely excluded
Conditional/Unclassified	More information is required for assessment
Unassessable/Unclassifiable	Available information is insufficient or contradictory and cannot be supplemented or verified

Causality assessment should consider the timing of drug exposure, clinical findings, dechallenge, rechallenge, concomitant medications and alternative explanations.

7.3 Severity Classification

Severity refers to the intensity of an adverse reaction.

It is generally classified into three categories:

- **Mild:** Symptoms are minor and usually do not require significant medical intervention.
- **Moderate:** Symptoms interfere with normal activities and may require medical treatment.
- **Severe:** Symptoms are intense, significantly affect normal functioning or require substantial medical intervention.

7.4 Seriousness Classification

Seriousness is a regulatory classification based on the outcome or consequences of an adverse event.

An event is considered serious if it:

1. Results in death.
2. Is life-threatening.
3. Requires inpatient hospitalisation or prolongation of existing hospitalisation.
4. Results in persistent or significant disability or incapacity.
5. Causes a congenital anomaly or birth defect.
6. Is another medically important event that may jeopardise the patient or require intervention to prevent one of the other serious outcomes.

Important distinction: Severity and seriousness are not interchangeable. A severe headache may be non-serious if it does not meet any regulatory seriousness criterion. Conversely, an event of moderate intensity may be serious if it requires hospitalisation.

7.5 Expectedness Assessment

Expectedness determines whether the reported reaction is consistent with the applicable reference safety information for the medicinal product.

- **Expected reaction:** The nature and severity of the reaction are consistent with the relevant reference safety information.
- **Unexpected reaction:** The nature, specificity or severity of the reaction is inconsistent with the applicable reference safety information.

Expectedness must be assessed against the appropriate reference document, such as the applicable Summary of Product Characteristics (SmPC), Investigator's Brochure or other regulatory reference safety information.

A reaction should not be labelled unexpected merely because it is uncommon or surprising to the reporter.

8. PRACTICAL CASE STUDY: COMPLETE ICSR ASSESSMENT

The following is a hypothetical case developed for training purposes to demonstrate the responsibilities of a Drug Safety Associate.

8.1 Case Scenario

A 54-year-old female patient visited a physician due to lower back pain. The physician prescribed Diclofenac 50 mg orally three times daily for pain management.

After five days of treatment, the patient developed severe abdominal pain, black tarry stools (melena), dizziness and weakness.

The patient was taken to the emergency department and admitted to the hospital. An upper gastrointestinal endoscopy revealed a bleeding gastric ulcer. Her haemoglobin level was found to be significantly reduced.

The treating physician suspected that the gastrointestinal bleeding was associated with diclofenac use. The medication was discontinued, and the patient received intravenous fluids, proton pump inhibitor treatment and a blood transfusion.

The patient remained hospitalised for three days and was recovering following treatment.

The physician subsequently reported the suspected adverse drug reaction to the pharmaceutical company.

8.2 Initial Case Information

Parameter	Case details
Case ID	PV-2026-001 (training ID)
Patient initials	A.B.
Age	54 years
Gender	Female
Suspected drug	Diclofenac
Dosage	50 mg
Route	Oral
Frequency	Three times daily
Indication	Lower back pain
Date of treatment initiation	10 June 2026
Date of adverse event onset	15 June 2026

Date of hospital admission	16 June 2026
Reporter	Treating physician
Relevant medical history	No known history of peptic ulcer disease
Concomitant medications	None reported
Treatment for ADR	Drug withdrawal, IV fluids, PPI and blood transfusion
Outcome	Recovering at the time of report

8.3 Identification of the Adverse Drug Reaction

The reported clinical manifestations include:

- Severe abdominal pain.
- Melena (black, tarry stools).
- Dizziness.
- Weakness.
- Bleeding gastric ulcer.
- Reduced haemoglobin level.

The endoscopic findings confirmed the presence of a bleeding gastric ulcer.

Diclofenac is a non-steroidal anti-inflammatory drug (NSAID). NSAIDs can cause gastrointestinal irritation, ulceration and gastrointestinal bleeding.

Therefore, the reported gastrointestinal bleeding is suspected to be associated with diclofenac administration.

Identified suspected adverse reaction: Gastrointestinal bleeding associated with a bleeding gastric ulcer.

8.4 Validation of the ICSR

The report must be evaluated against the four minimum validity criteria.

Validity criterion	Information available	Status
Identifiable patient	54-year-old female, initials A.B.	Fulfilled
Identifiable reporter	Treating physician	Fulfilled
Suspected medicinal product	Diclofenac 50 mg	Fulfilled
Suspected adverse event	Bleeding gastric ulcer and gastrointestinal bleeding	Fulfilled

Valid ICSR

Conclusion: All four minimum criteria are fulfilled. Therefore, the report qualifies as a valid ICSR and can proceed to the next stages of case processing.

8.5 Day 0 Assessment

For this hypothetical case, assume that the pharmaceutical company first received the valid report on 16 June 2026.

Therefore:

- Initial receipt date: 16 June 2026.
- Day 0: 16 June 2026.
- Regulatory clock: Starts according to the applicable reporting requirements from this receipt date.

The company should record the date accurately because it determines the applicable regulatory submission deadline.

8.6 Duplicate Case Assessment

The Drug Safety Associate must search the safety database to determine whether the same adverse event has already been reported.

The search may use:

- Patient initials.
- Age and gender.
- Suspected medicinal product.
- Adverse event.
- Reporter details.
- Event and treatment dates.

For this hypothetical case, assume that no matching previous report is identified.

Duplicate status: No duplicate identified.

8.7 Medical Coding of the Case

Medical coding is performed using standardised medical terminology.

The following are illustrative MedDRA coding terms. Final coding should be verified against the current licensed MedDRA version and the source documentation.

Reported information	Illustrative MedDRA term
Black tarry stools	Melena
Bleeding gastric ulcer	Gastric ulcer haemorrhage
Gastrointestinal bleeding	Gastrointestinal haemorrhage
Abdominal pain	Abdominal pain
Dizziness	Dizziness

Weakness	Asthenia

The suspected medicinal product is Diclofenac, which should be coded using the applicable WHO Drug Dictionary or company-approved drug dictionary.

Importance of coding: Standardised terminology allows adverse events to be analysed, compared and monitored across different reports and geographical regions.

8.8 Seriousness Assessment

The patient required inpatient hospitalisation for three days due to gastrointestinal bleeding.

According to pharmacovigilance seriousness criteria, an adverse event requiring inpatient hospitalisation is classified as serious.

Seriousness criterion	**Assessment**
Death	No
Life-threatening	Not documented
Hospitalisation	Yes
Persistent disability	No
Congenital anomaly	Not applicable
Other medically important event	Not separately established

Serious adverse event

Conclusion: The case is classified as serious because it resulted in inpatient hospitalisation.

The available information does not establish that the patient was life-threateningly ill. Hospitalisation alone is sufficient to meet a seriousness criterion.

8.9 Severity Assessment

Severity is assessed according to the intensity of the reported clinical manifestations and the extent of medical intervention required.

In this case:

- The patient experienced significant abdominal pain and gastrointestinal bleeding.
- Haemoglobin levels were reduced.
- Endoscopy confirmed a bleeding gastric ulcer.
- Hospitalisation was required.
- Blood transfusion and medical treatment were administered.

Severe

Conclusion: The adverse reaction is classified as severe due to its significant clinical intensity and the requirement for substantial medical intervention.

8.10 Causality Assessment Using WHO-UMC

The WHO-UMC system is used to evaluate the possible relationship between a suspected drug and an adverse event.

For this case, the following factors are considered:

Assessment factor	Case observation
Temporal relationship	The reaction developed after five days of diclofenac administration
Known pharmacological effect	Diclofenac is associated with gastrointestinal ulceration and bleeding
Dechallenge	Diclofenac was discontinued
Response to withdrawal	The patient was recovering following treatment and withdrawal
Rechallenge	Not performed
Alternative causes	No previous peptic ulcer disease or concomitant medication was reported
Clinical evidence	Endoscopy confirmed a bleeding gastric ulcer

WHO-UMC causality category: Probable/Likely (provisional assessment).

Justification:

- The onset of the reaction followed diclofenac administration within a clinically plausible time.
- Gastrointestinal ulceration and bleeding are recognised risks associated with diclofenac.
- The patient improved following drug discontinuation and appropriate treatment.
- No significant alternative cause was identified in the information available.

Rechallenge was not performed, which is appropriate given the potential risk of recurrent gastrointestinal bleeding. A rechallenge is not required for a probable/likely classification.

This is a training assessment based on the hypothetical information, not a definitive medical diagnosis.

8.11 Expectedness Assessment

Expectedness is assessed by comparing the reported adverse reaction with the applicable reference safety information for the suspected medicinal product.

Diclofenac is known to be associated with gastrointestinal adverse effects, including peptic ulceration and gastrointestinal haemorrhage.

Assuming that the applicable reference safety information for the specific diclofenac product includes gastric ulceration and gastrointestinal bleeding, the reaction is classified as:

Expected

Justification: The nature of the reported reaction is consistent with the known gastrointestinal safety profile of diclofenac.

The actual expectedness decision must be confirmed against the relevant current product information, not based solely on general pharmacological knowledge.

8.12 Dechallenge and Rechallenge Assessment

Dechallenge: Dechallenge refers to discontinuation of the suspected medicinal product following an adverse event to assess whether the reaction improves.

In this case, diclofenac was discontinued, and the patient showed improvement following medical treatment.

Rechallenge: Rechallenge refers to reintroducing the suspected medicinal product after the adverse event to determine whether the reaction occurs again.

In this case, rechallenge was not performed because re-exposure could cause recurrent gastrointestinal bleeding.

8.13 Final Pharmacovigilance Case Classification

Parameter	**Final assessment**
Case validity	Valid
Duplicate status	No duplicate identified
Suspected drug	Diclofenac
Adverse reaction	Gastrointestinal bleeding associated with bleeding gastric ulcer
Severity	Severe
Seriousness	Serious
Seriousness criterion	Inpatient hospitalisation
Causality	Probable/Likely (WHO-UMC)
Expectedness	Expected, subject to reference safety information verification
Dechallenge	Positive, with improvement after withdrawal and treatment
Rechallenge	Not performed

Patient outcome	Recovering
Regulatory reporting	Report according to applicable serious ICSR requirements

9. SAMPLE ICSR CASE NARRATIVE

Case Narrative:

A 54-year-old female patient was prescribed diclofenac 50 mg orally three times daily for the treatment of lower back pain on 10 June 2026.

After five days of treatment, on 15 June 2026, the patient developed severe abdominal pain, melena, dizziness and weakness.

On 16 June 2026, the patient was admitted to the hospital. An upper gastrointestinal endoscopy revealed a bleeding gastric ulcer, and laboratory investigations showed a significantly reduced haemoglobin level.

The treating physician suspected an association between diclofenac administration and the reported gastrointestinal bleeding. Diclofenac was discontinued, and the patient received intravenous fluids, proton pump inhibitor therapy and a blood transfusion.

The patient remained hospitalised for three days and was recovering following treatment at the time of reporting.

The case was classified as serious due to inpatient hospitalisation and severe based on the clinical intensity and medical intervention required. The causality assessment using the WHO-UMC system was provisionally classified as Probable/Likely. The reaction was considered expected, subject to confirmation against the applicable reference safety information.

10. REGULATORY REPORTING AND SUBMISSION

After completion of data entry, quality review and medical assessment, the case should be prepared for regulatory submission.

Since the hypothetical patient required inpatient hospitalisation, the case meets the seriousness criterion and should be prioritised for reporting.

For an Indian marketing authorisation holder, the applicable CDSCO requirements must be checked. The CDSCO 2024 PSUR guidance specifies a 15-calendar-day reporting timeline for serious adverse events covered by that guidance.

The submission process involves:

1. Verification of the case's validity.
2. Confirmation of the seriousness classification.
3. Verification of the reporting timeline.
4. Review of all patient and drug information.
5. Verification of MedDRA and drug coding.

6. Final review of the case narrative.
7. Preparation of the electronic ICSR.
8. Submission through the applicable regulatory reporting channel.
9. Monitoring the submission acknowledgement.
10. Documenting any follow-up information received after submission.

If additional information becomes available, a follow-up report should be prepared and submitted according to the applicable requirements.

11. ROLE AND RESPONSIBILITIES OF A DRUG SAFETY ASSOCIATE

A Drug Safety Associate plays an important role in the pharmacovigilance process.

The major responsibilities include:

a. Case Intake and Triage: Receiving adverse event reports and checking whether they fulfil the minimum validity criteria.
b. Data Entry: Entering accurate patient, medicinal product and adverse event information into the safety database.
c. Medical Coding: Coding adverse events using MedDRA and medicinal products using an approved drug dictionary.
d. Case Narrative Writing: Preparing a clear and chronological summary of the patient's clinical experience.
e. Query Management: Contacting the reporter or relevant source to obtain missing information.
f. Quality Control: Checking the accuracy, consistency and completeness of case information.
g. Regulatory Compliance: Ensuring that the case is processed and submitted within the applicable regulatory timelines.
h. Follow-up Management: Obtaining additional information regarding treatment, investigations, patient outcome and other relevant details.
i. Documentation: Maintaining accurate records of case processing, assessments, corrections and submissions.

12. IMPORTANCE OF ICSR IN PATIENT SAFETY

ICSRs play an essential role in post-marketing drug safety monitoring.

Their importance includes:

- Early identification of previously unknown adverse drug reactions.
- Detection of potential safety signals.
- Identification of risk factors associated with medicinal products.
- Continuous monitoring of the benefit-risk profile of medicines.
- Supporting regulatory decisions and safety warnings.
- Improving prescribing practices and patient counselling.
- Promoting the safe and rational use of medicines.

13. CONCLUSION

Individual Case Safety Reports are an essential component of pharmacovigilance because they provide structured information about suspected adverse events associated with medicinal products.

The ICSR case processing workflow includes case receipt, triage, validation, duplicate checking, data entry, medical coding, narrative preparation, quality review, medical review and regulatory submission.

Through the hypothetical diclofenac case, the practical application of pharmacovigilance principles was demonstrated. The report fulfilled all four minimum ICSR validity criteria. The adverse reaction was assessed as severe and serious due to inpatient hospitalisation. The WHO-UMC causality assessment was provisionally classified as Probable/Likely, while expectedness was considered expected subject to verification against the product's reference safety information.

A Drug Safety Associate must ensure that every case is processed accurately, consistently and within the applicable regulatory timelines. Effective ICSR processing ultimately contributes to improved medication safety and better patient outcomes.

14. REFERENCES

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